KEY BENEFIT ADMINISTRATORS

OPPORTUNITY REVIEW | NOVEMBER 2026

Direct-to-Employer Go-to-Market Strategy

Exploring an OhioHealth direct-to-employer health plan within Ohio's statewide Key Preferred Network

Prepared for

OhioHealth

Statewide network

KEY PREFERRED NETWORK



EXECUTIVE SUMMARY

Ohio is ready for a statewide direct-to-employer network

A statewide network is forming

KBA is already working with University Hospitals in Northeast Ohio and The Christ Hospital in Southwest Ohio on a statewide plan. OhioHealth completes Central and Southeast Ohio.

Two ways for OhioHealth to participate

Launch an OhioHealth-branded plan built on Key Preferred's broad statewide contracts, or join Key Preferred as the preferred health system for its region.

Steerage without restricting choice

Key Preferred Benefits reward members for using preferred systems, while the broad network lets them go anywhere. KBA networks average 83% Tier 1 steerage.

$20.6M

Projected Year 4 net margin change for OhioHealth
Target case for an OhioHealth-branded plan: 18,600 members, 67% inpatient and 58% outpatient steerage, 26.4% commercial margin



ABOUT KEY BENEFIT ADMINISTRATORS

A national leader in self-funded plans

1979
Founded in Indianapolis

3,000+
Corporate clients

3M
Members under management

700
Employees

- Largest independently owned full-service third-party administrator
- Specialists in self-funded plan administration and population health management
- A leader in the direct-to-employer movement
- Works with every professional team: agents, networks and MGAs
- The Key Family of Companies is a vertically integrated group of benefit organizations

OUR FOUNDING AND HERITAGE

46 years of striving for honesty in healthcare



Larry Dust
Founder, Key Benefit Administrators

1979

Founded KBA

After co-founding a life insurance agency in 1973 and Key Life Insurance in 1976, Larry Dust launched KBA as a TPA focused on honest healthcare delivery.

1985

Partnered with BCBS

Sold to Blue Cross Blue Shield of Indiana, gaining capital and market access while keeping operational independence.

1992

Reacquired KBA

Larry bought the business back, returning it to full independence and a patient-focused mission.

Today

Largest independent TPA

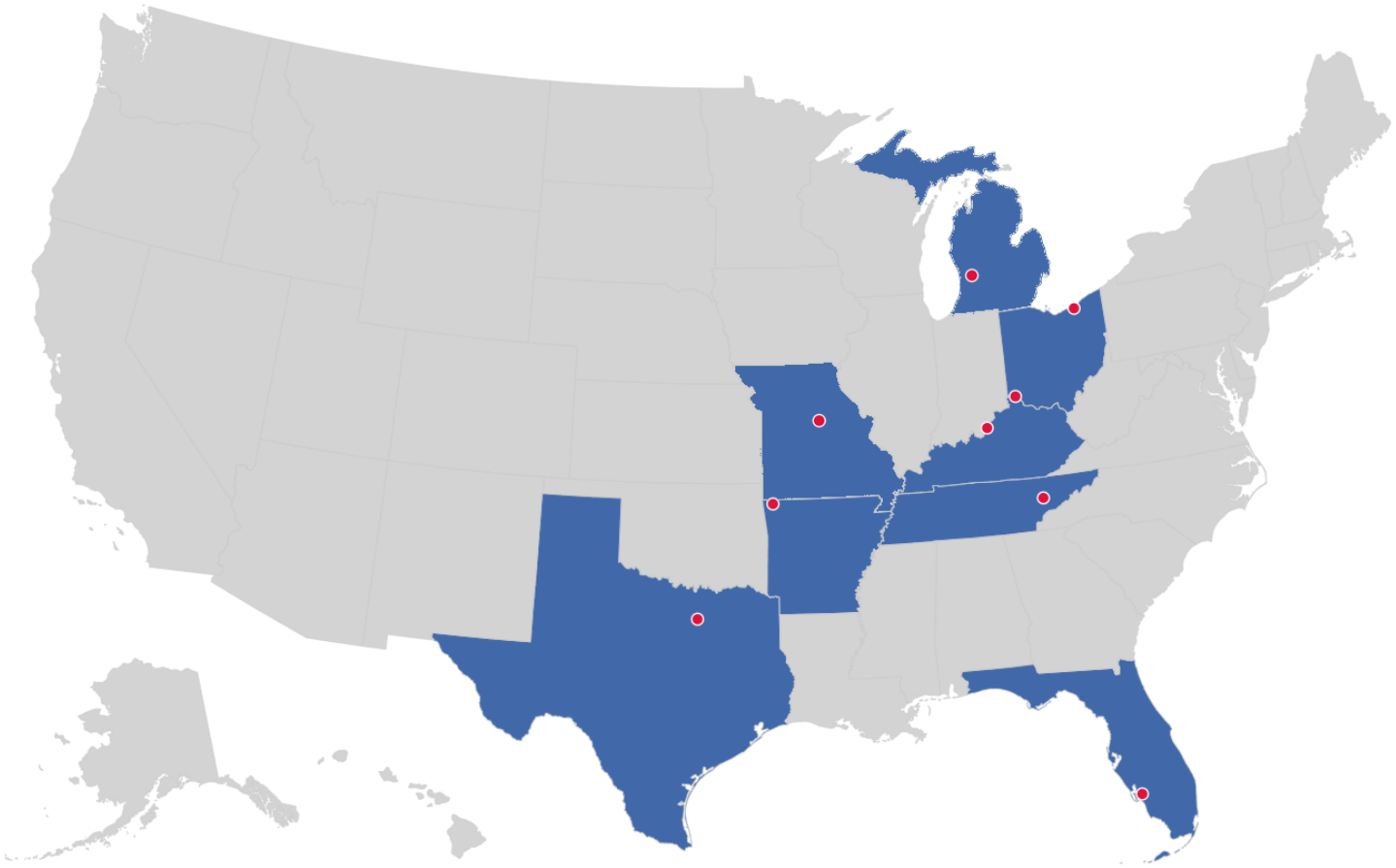
KBA is the country's largest independently owned TPA, managing millions of members across diverse markets.

Never forgotten who gives healthcare

KBA has never participated in reference-based pricing, does not take refunds, and delivers healthcare at 11% below market rates. Our commitment remains with providers and patients, not profit extraction.

TRACK RECORD

Market leader in private-labeled DTE plans



Private-labeled DTE markets

- **Cleveland, OH**
- **Cincinnati, OH**
- Grand Rapids, MI
- Columbia, MO
- Louisville, KY
- Knoxville, TN
- Bentonville, AR
- Dallas-Fort Worth, TX
- Fort Myers, FL

Already active in two Ohio markets

01

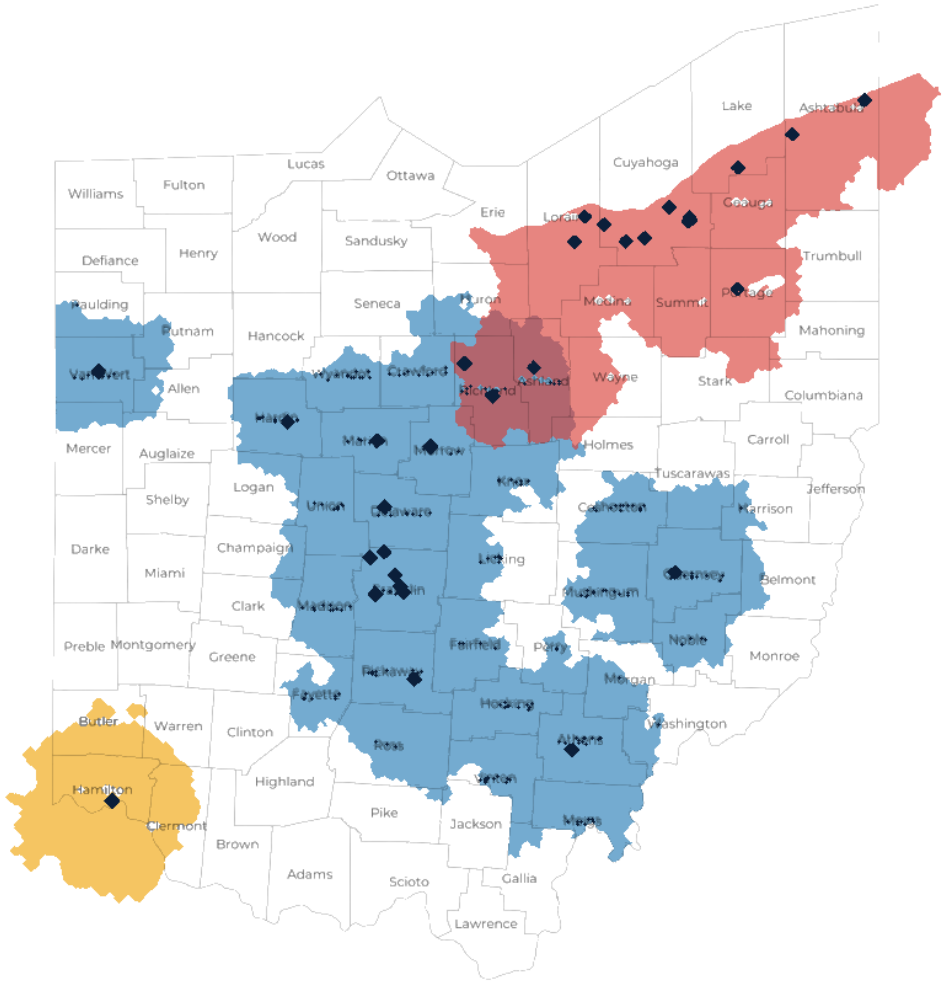
Ohio is different: one statewide network, two ways in

How OhioHealth fits alongside University Hospitals and The Christ Hospital in the Key Preferred Network

KEY PREFERRED NETWORK

KEY BENEFIT ADMINISTRATORS

Three anchor systems, three largest Ohio metros



University Hospitals — WORKING WITH KBA TODAY

Northeast Ohio: Cleveland, Akron and the Lake Erie corridor to Ashtabula

OhioHealth — THE OPPORTUNITY

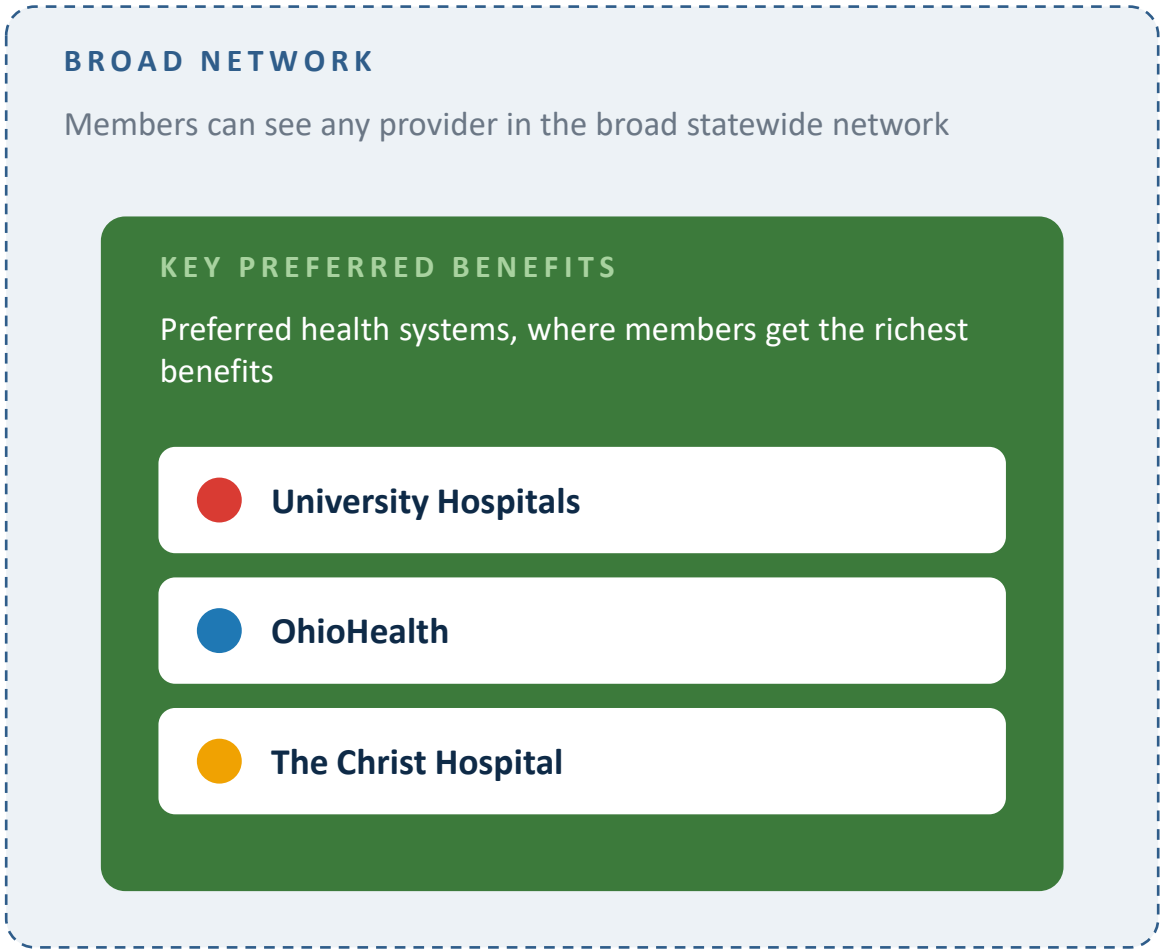
Central and Southeast Ohio: Columbus, Mansfield, Marion, Athens, Cambridge and Van Wert

The Christ Hospital Health Network — WORKING WITH KBA TODAY

Southwest Ohio: Greater Cincinnati

Complementary footprints. The systems overlap only around Mansfield and Ashland, so each can be the clear preferred choice in its own region. Toledo, Dayton and Canton-Youngstown remain open for future preferred partners.

Not a typical PPO: steer to preferred, keep choice



Steer with rewards, not restrictions

Preferred Plus Benefits, the Health Guardian Account and a 0% Convenience Card can take a member's out-of-pocket cost to $0 at a preferred system.

Keep freedom of choice

Members can still use any provider in the broad network, which makes the plan easy for employers and brokers to say yes to.

EPO-like steerage inside a PPO

KBA's DTE networks average 83% Tier 1 steerage, against OhioHealth's estimated 34% inpatient market share today.

YOUR CHOICE

Two ways for OhioHealth to participate

PATH A

An OhioHealth-branded plan, powered by Key Preferred

HOW IT WORKS	OhioHealth launches its own DTE plan as the preferred tier. Key Preferred's broad statewide contracts give members access everywhere else.
BRAND	OhioHealth names the plan and owns the employer relationship.
ECONOMICS	OhioHealth sets the fees and keeps the plan profits.
BEST FIT IF	OhioHealth wants a flagship branded product and full control.

PATH B

Join Key Preferred as a preferred system

HOW IT WORKS	OhioHealth becomes the preferred system for Central and Southeast Ohio inside the statewide plan, next to UH and The Christ Hospital.
BRAND	OhioHealth is featured as a preferred system in a statewide plan.
ECONOMICS	Steered volume from every Key Preferred employer with members in OhioHealth's region.
BEST FIT IF	OhioHealth wants statewide employer reach and a faster start on an existing plan.

The financial projections in this deck model Path A. Both paths use the same KBA administration, Key Preferred Benefits and statewide broad network.

WHY STATEWIDE MATTERS

A statewide plan opens doors a regional plan cannot

Employers with Ohio-wide workforces

A Columbus employer with staff in Cleveland and Cincinnati can offer one plan with a preferred system in every region.

Volume from outside the home market

Employers headquartered in Cleveland or Cincinnati steer their Central Ohio employees to OhioHealth.

One product for brokers

Brokers sell a single statewide offering instead of three regional plans, which shortens the sales cycle.

A stronger competitive position

Three respected systems together give employers a credible alternative to national carrier networks across Ohio.

02

The direct-to-employer model

A health plan built on a direct relationship with self-funded employers in OhioHealth's market

KEY BENEFIT ADMINISTRATORS

DIRECT-TO-EMPLOYER (DTE)

Your health plan, your market, your margin

- **YOU** pick the name of the health plan
- **YOU** determine the fees charged
- **YOU** practice the medicine
- **YOU** receive the profits
- **WE** make it successful

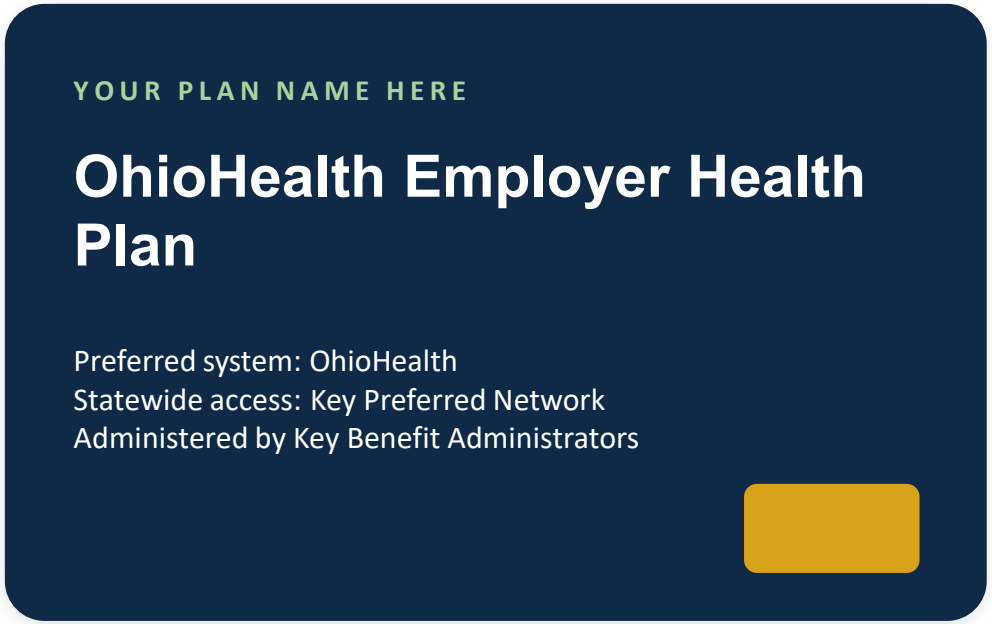


Illustration only. Under Path A, OhioHealth chooses the plan name and brand.



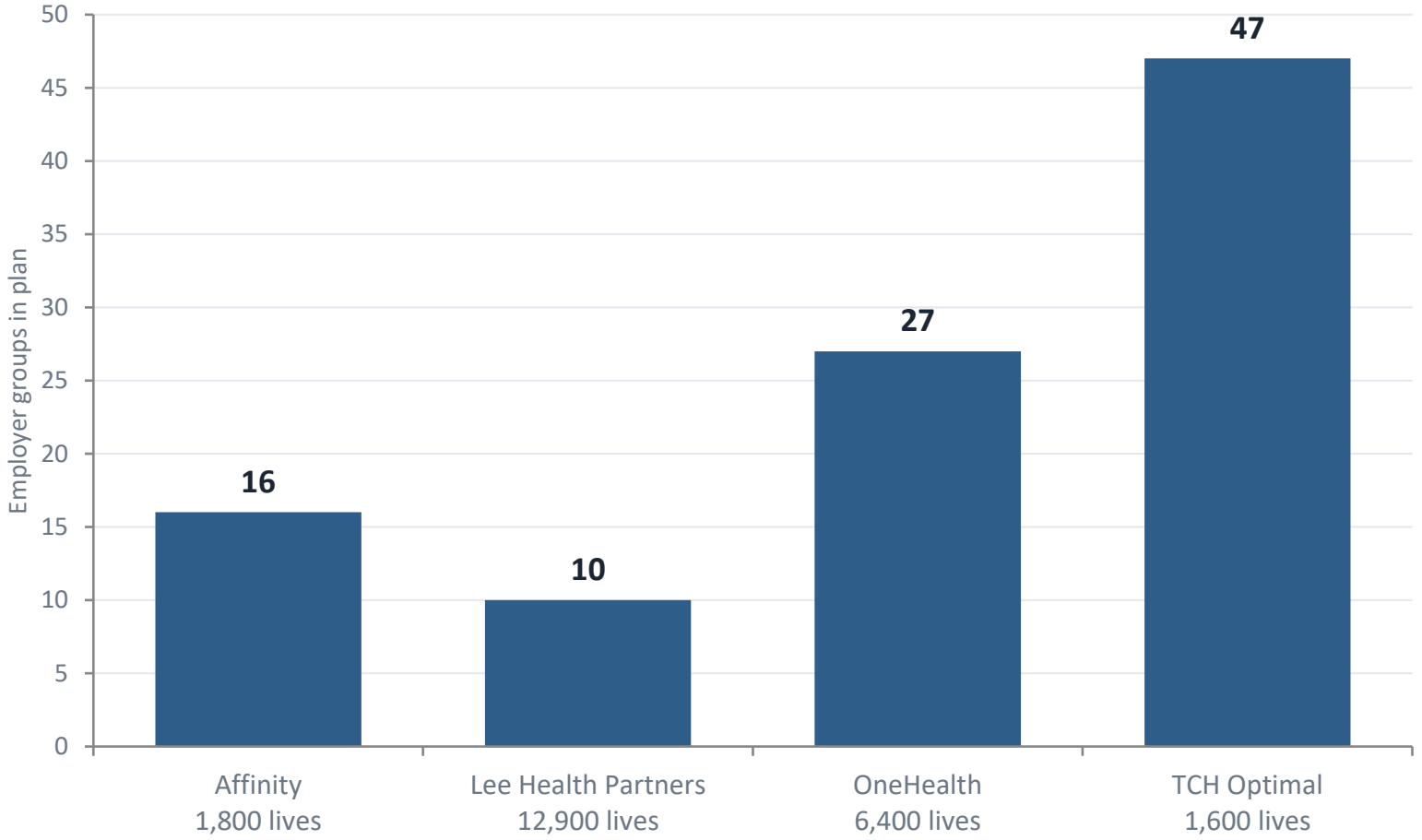
DIRECT-TO-EMPLOYER STRATEGY

Powerful employer value

- Lower medical spend through lower unit costs and better integrated care
- A seat "at the table" with OhioHealth
- Aligned financial incentives
- Complete transparency into plan expenses and cost drivers
- Aggressive stop-loss premiums and policy terms
- Higher employee satisfaction
- Better cash flow and a more stable future cost trend

PROVEN ADOPTION

KBA plans average 25 employer groups in two years



25

average employer groups per plan within the first two years

22,700

lives across these four plans in their first two years

Only available to premier health systems

OhioHealth

OhioHealth meets every criterion

STRONG

Brand quality

EXTENSIVE

Patient access

AFFORDABLE

Value pricing

EXCELLENT

Care quality

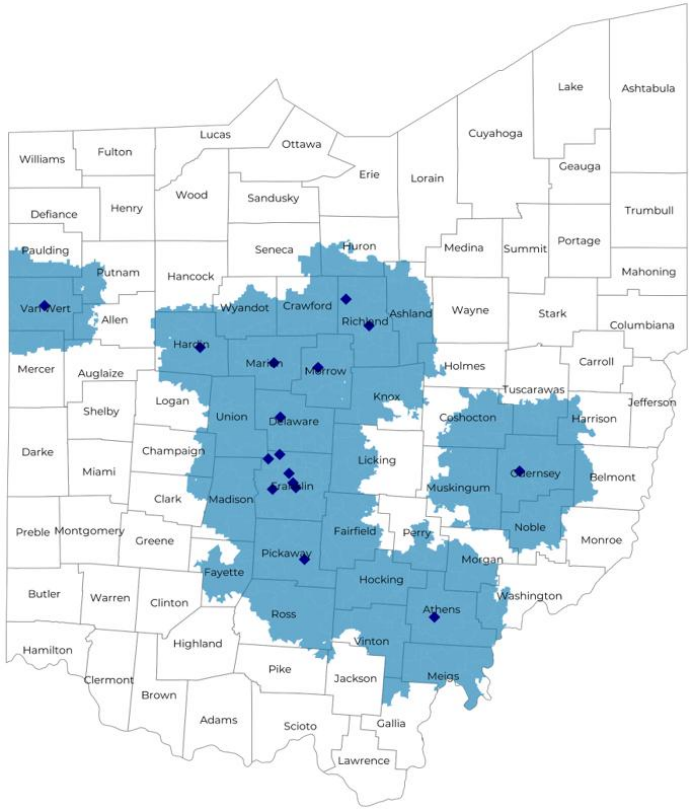
03

Market opportunity and projections

What an OhioHealth-branded plan could deliver in OhioHealth's service area

KEY BENEFIT ADMINISTRATORS

Significant room for plan growth



3,500+

Employers

800,000+

Employees

1,500,000+

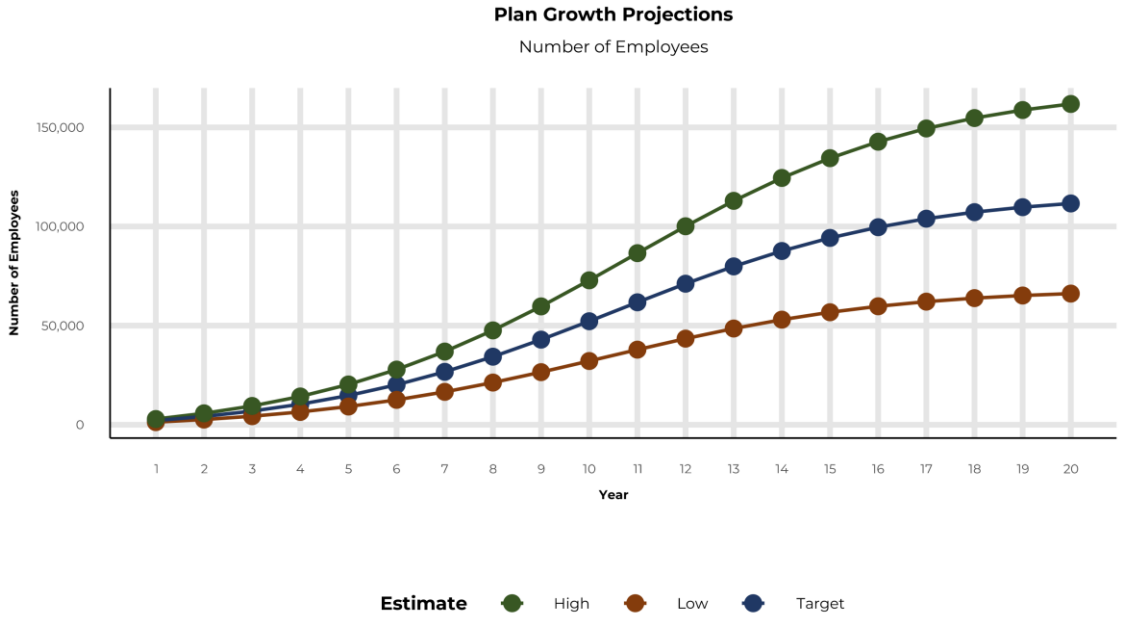
Members

Commercial employers headquartered in the OhioHealth catchment area, by employer size (number of employees)

	50-100	101-200	201-400	401-1,000	1,001-3,000	3,000+
Employers	1,995	900	565	300	111	79
Employees	149,600	135,000	169,500	210,000	222,000	474,000
Members	269,300	243,000	305,100	378,000	399,600	853,200

Plan potential growth paths

Population	2,585K
Working age	1,546K
Insured	1,091K
Self-funded	765K



Service area includes Allen, Ashland, Athens, Champaign, Coshocton, Crawford, Delaware, Fairfield, Franklin, Guernsey, Hardin, Harrison, Hocking, Huron, Knox, Licking, Madison, Marion, Meigs, Mercer, Morgan, Morrow, Muskingum, Noble, Perry, Pickaway, Richland, Ross, Tuscarawas, Union, Van Wert, Vinton, Washington and Wyandot counties.

LONG-TERM ECONOMICS

Growing powerful, lasting profitability

Early stage

Gaining name recognition, early growth

New revenue
$40.28M

New profit
$10.66M

Growth stage

Strong reputation, strong growth

New revenue
$246.18M

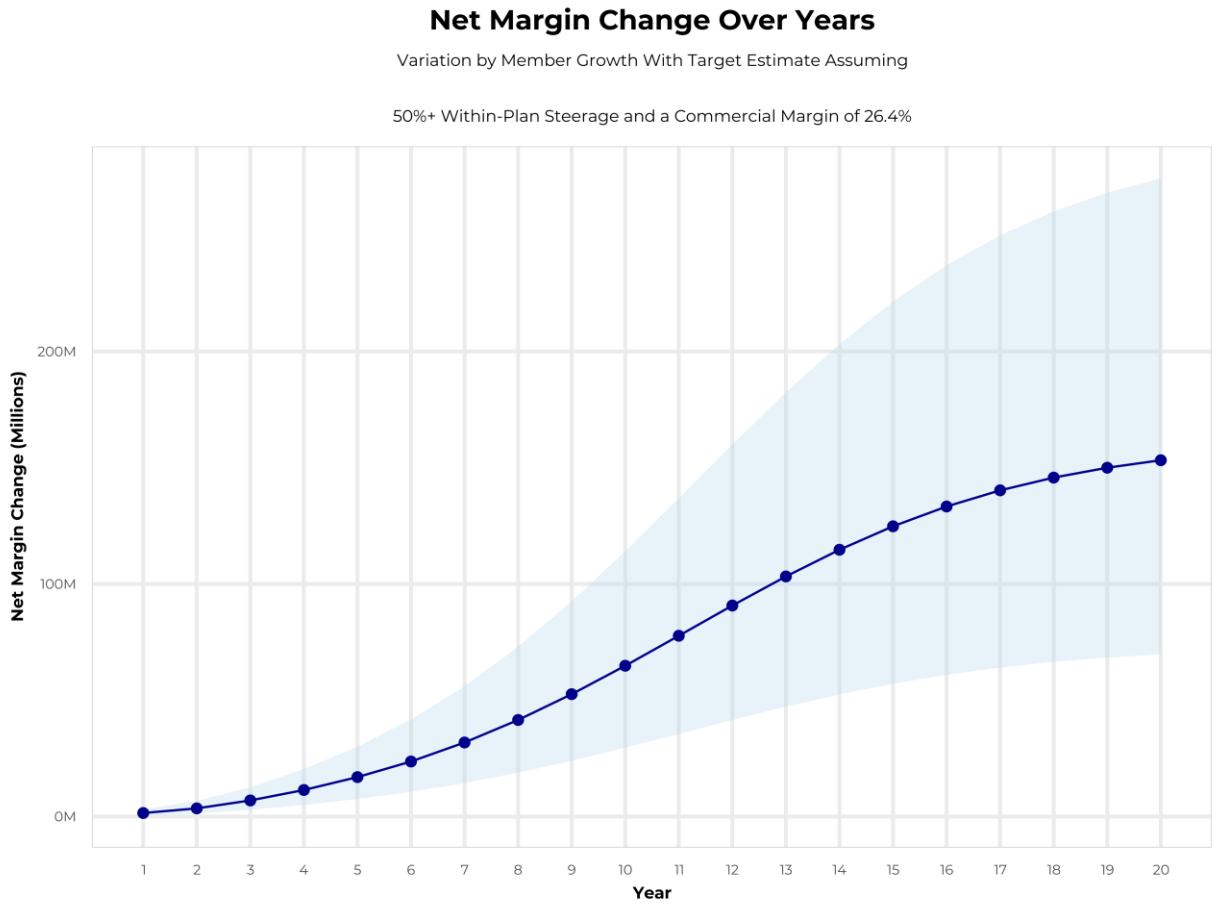
New profit
$66.08M

Mature stage

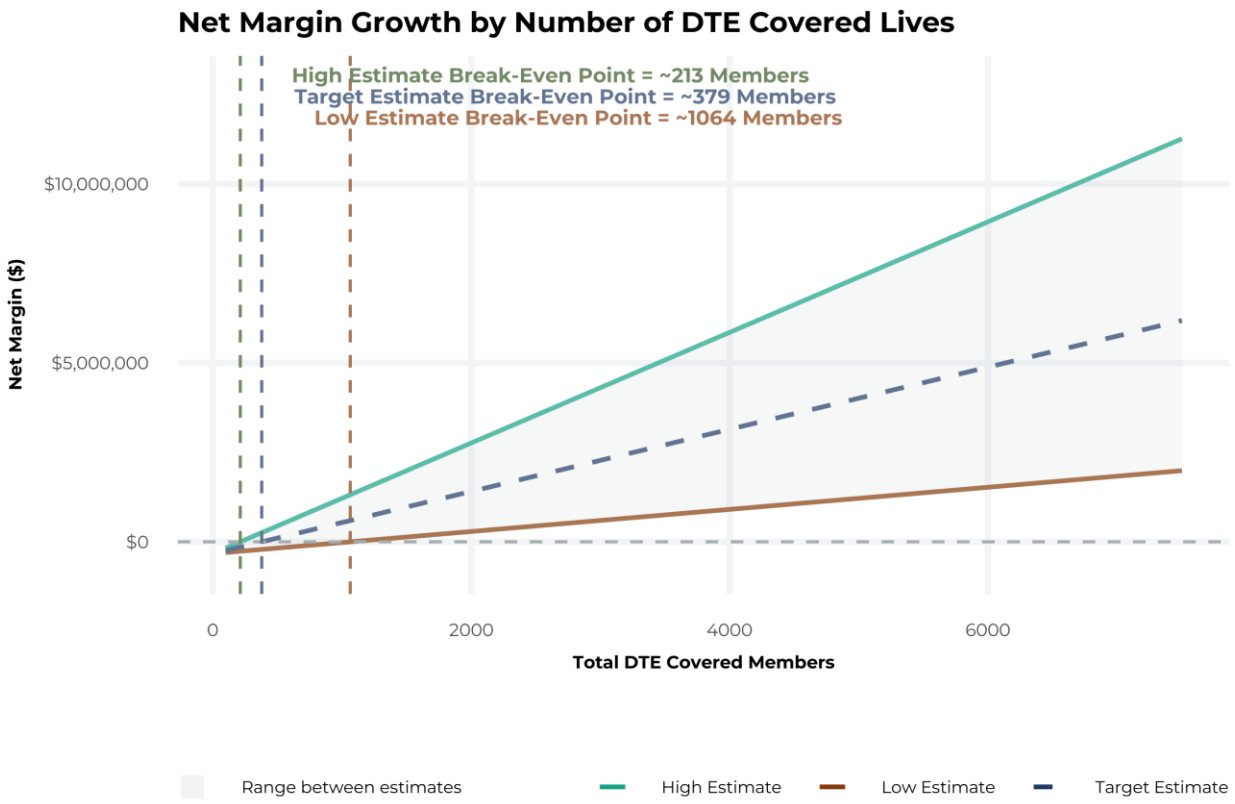
High profitability, market saturation

New revenue
$512.03M

New profit
$137.44M



Model assumptions and break-even



Patient steerage

KBA estimates OhioHealth has 34% inpatient and 22% outpatient market share in its service area. The tiered DTE design creates immediate steerage, with more over time from member bonus benefits and the Convenience Card.

Low	High
10% below target	**10% above target**

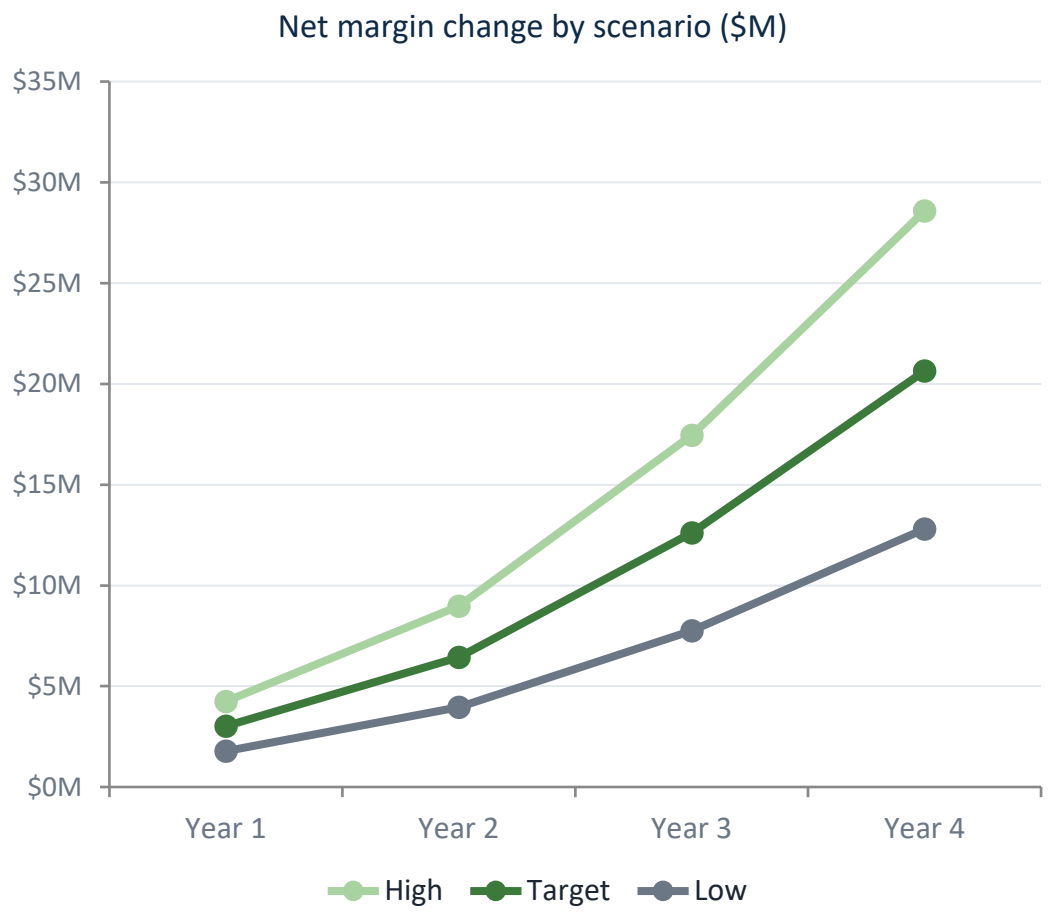
Commercial margin

KBA observes a 33% OhioHealth commercial margin. We conservatively model DTE pricing 10% below market, offset by a 10 point gain from friendlier billing and better patient collections.

Low	Target	High
21.4%	**26.4%**	**31.4%**

FOUR-YEAR PROJECTIONS

Net margin change grows to $20.6M by Year 4



Target case ($K)	Year 1	Year 2	Year 3	Year 4
Number of Members	3,800	7,500	12,400	18,600
Incremental Margin	3,301.4K	6,518.9K	12,640.4K	20,592.0K
Network Access Fee	45.6K	90.0K	148.8K	223.2K
Implementation Fee (One-Time)	-150.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-180.0K	-180.0K	-180.0K	-180.0K
Net Margin Change	**3,017.0K**	**6,428.9K**	**12,609.2K**	**20,635.2K**

HIGH CASE, YEAR 4

$28.6M

25,700 members

LOW CASE, YEAR 4

$12.8M

11,600 members

SINGLE-YEAR SUMMARY

Margin impact by number of covered members

Target estimate: 50%+ within-plan steerage and a 26.4% commercial margin

DTE Members	Pre-DTE Revenue	Pre-DTE Margin	DTE Revenue	DTE Incr. Revenue	DTE Total Margin	DTE Incr. Margin	Net Margin Less DTE Fees
100	$294K	$78K	$618K	$325K	$163K	$86K	**$-243K**
500	$1,468K	$388K	$3,091K	$1,623K	$816K	$428K	**$104K**
1,000	$2,936K	$775K	$6,182K	$3,245K	$1,632K	$857K	**$539K**
2,500	$7,340K	$1,938K	$15,454K	$8,114K	$4,080K	$2,142K	**$1,842K**
10,000	$29,362K	$7,752K	$61,816K	$32,454K	$16,320K	$8,568K	**$8,358K**
20,000	$58,724K	$15,503K	$123,633K	$64,909K	$32,639K	$17,136K	**$17,046K**



Upside not included: 340B. Projections exclude 340B contribution margin, which could reach $136 PMPM by optimizing 340B through Rx plan design, OhioHealth's approved sites with network integration, and an OhioHealth proprietary PBM program. Detailed analysis is in the appendix.

04

Steering members to OhioHealth

The Key Incentive Plan turns preferred-provider choice into real dollars for members

KEY BENEFIT ADMINISTRATORS

Affordability without compromise

Employers take back control of their healthcare programs while members keep freedom of choice and are rewarded for choosing high-quality, cost-effective care.

Basic medical benefit

Customized major medical plan with access to a broad network of providers.

Preferred Plus Benefits*

Benefits paid at preferred providers: real dollars to members who get the care they need, when they need it.

Health Guardian Account

KBA tracks and administers Preferred Plus Benefits, out-of-pocket expenses and the Convenience Card.

Healthcare Convenience Card

0% interest, no-fee card for out-of-pocket expenses at preferred providers.

Chronic disease and acute illness management

Patented program covering 28 chronic illnesses with 11% to 17% savings and a $14 to $1 ROI.

PBM, specialty Rx and GLP-1 program

Transparent Rx programs with 1-on-1 coaching: about 20%+ savings on Rx and 10% on overall claims.

*Daily inpatient hospital benefit, OP surgery/anesthesia, OP radiology/X-ray, OP diagnostic labs, health screening benefit, wellness benefit, critical illness

Base medical + Preferred Plus: the deductible can vanish

Example: a member with a $2,500 deductible and $4,500 out-of-pocket maximum has a heart attack and cardiac surgery with a 5-day inpatient stay. At a preferred provider, $4,925 in Preferred Plus Benefits cover the full $4,500 out-of-pocket, so the member owes $0. At a non-preferred provider, the member owes $4,500.

Base medical benefits*	
Deductible (single / family)	$2,500 / $4,500
Coinsurance	80% / 20%
Max out-of-pocket (single / family)	$4,500 / $9,000
Chronic disease regimens of care	Covered at 100%
Healthcare Convenience Card	0% interest
Health Guardian Account	Included

Preferred Plus Benefits	Preferred	Non-preferred
OP radiology / X-ray	$200	$0
OP diagnostic labs	$100	$0
Preferred hospital stay ($500/day, 5 days)	$2,500	$0
Critical illness benefit	$2,000	$0
Health screening benefit	$125	$0
Total employee out-of-pocket	$4,500	$4,500
Preferred Plus Benefits	$4,925	$0
Employee amount due to provider	**$0**	**$4,500**
Added to Health Guardian Account	$425	$0

*Base medical benefits are for illustration only and can be customized to client needs.



WHAT WILL THIS CLAIM COST ME?

The benefit steers the patient to OhioHealth

Explanation of benefits: heart attack, cardiac surgery	
THE CHARGES	
Medical bills	$25,000
Out-of-pocket due to provider	$4,500
THE CREDITS	
OP radiology / X-ray	$200
OP diagnostic labs	$100
Preferred hospital stay ($500/day, 5 days)	$2,500
Critical illness benefit	$2,000
Health screening benefit	$125
Benefit total	**$4,925**
Out-of-pocket paid to hospital	($4,500)
Balance paid to Health Guardian Account	**$425**

OhioHealth patients can have all out-of-pocket liability reduced to $0

KEY BENEFIT ADMINISTRATORS



THE KBA HEALTHCARE CONVENIENCE CARD

Creating EPO steerage in a PPO network

The card works only for specific merchant codes, so it rewards care at preferred providers.

1. **Use at specific healthcare providers**
   Designated hospitals, urgent care facilities, physicians and eligible ancillary providers
2. **Out-of-pocket expenses qualify**
3. **0% interest rate**
4. **Employee sets the repayment schedule**
5. **Simple to qualify**
   Answer one simple question

WEIGHT MANAGEMENT

GLP-1s that don't break the bank

Prime Therapeutics reports GLP-1 spend per member per month tripled by March 2025, now about 15% of total pharmacy spend, with more than 1 in 50 members filing a GLP-1 claim in Q1 2025.

THE CHALLENGE

$1,089-$1,349

per month for traditional GLP-1s, an unsustainable cost that limits employee access

OUR SOLUTION

$249

per month for generic GLP-1 medications secured by Key Benefits, with strict budget control

THE IMPACT

$7M

saved by one major city that was spending $11M a year on GLP-1s

Beyond medication: comprehensive care

Members get specialized coaching twice a month on keeping muscle strength, moderate exercise and lasting lifestyle changes that sustain weight loss after GLP-1 treatment ends.

270

conditions linked to obesity (Cleveland Clinic)

$8.5K

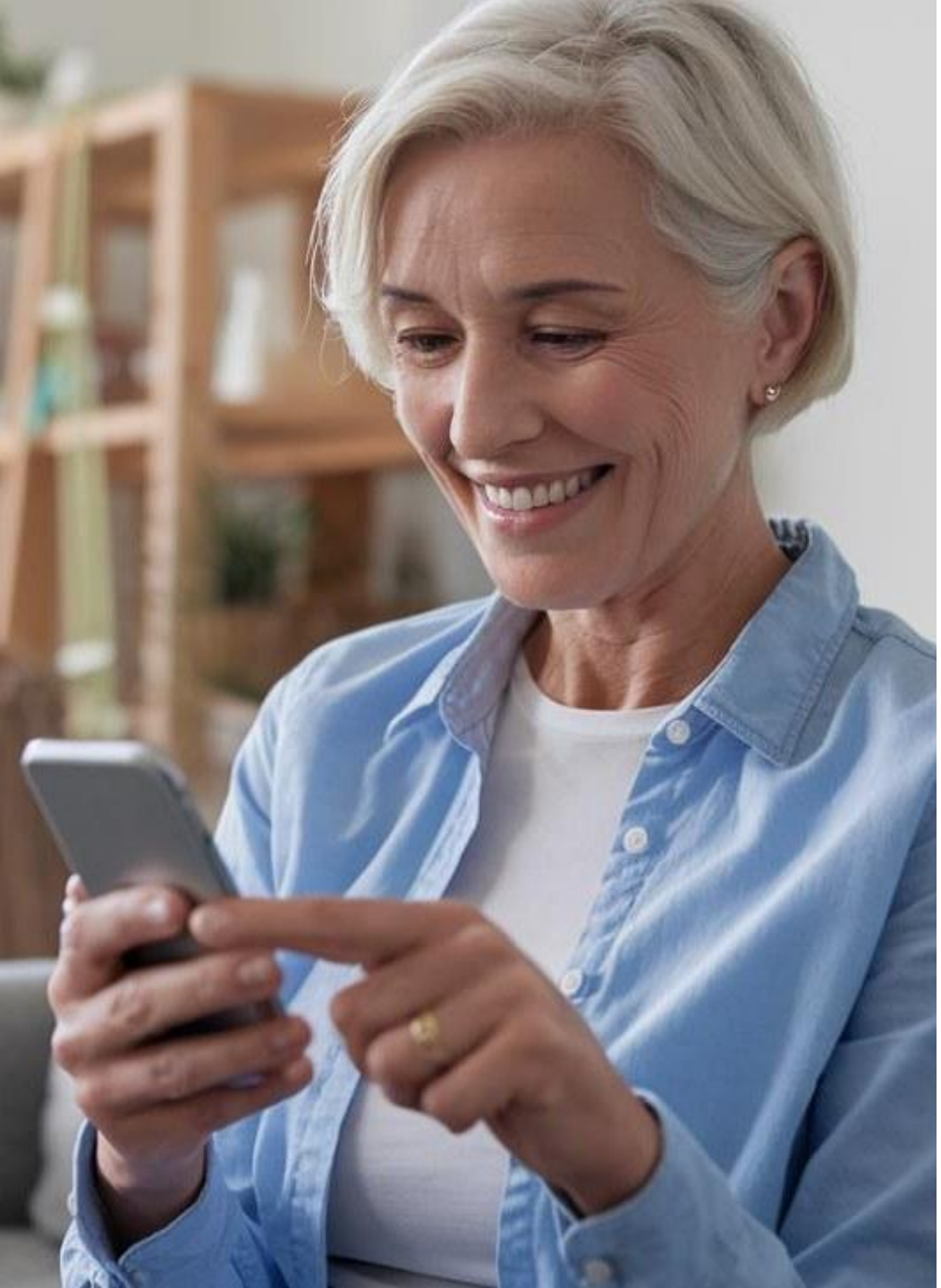
added annual cost per obese individual (KFF)

05

Platform and go-to-market

Clinical integration, pharmacy, stop-loss, sales and implementation

KEY BENEFIT ADMINISTRATORS



CLINICAL INTEGRATION

Integration with the Epic EHR

KBA shares evidence-based standards of care for **27 chronic conditions** directly into Epic, so OhioHealth clinicians see gaps in care at the point of care.

Example standard: Hyperthyroidism: yearly clinical evaluation, TSH and T4

Epic App Orchard

VALUE-BASED CARE

The missing piece to value-based care



Patient engagement is the weak link in value-based care. OhioHealth's clinical leadership and employer participation can drive better outcomes.



KBA's proactive care management team supplements OhioHealth's own outreach to patients.



OhioHealth excels in standards-of-care completion for the chronically ill, improving care and cost.



Integrating OhioHealth's managed care with KBA's programs positions OhioHealth as the value leader.

Transparent PBM integration

OhioHealth-centered pharmacy

- Support OhioHealth's retail pharmacies
- Optimize OhioHealth's 340B drug pricing program
- Option for OhioHealth to create its own PBM program
- Custom phone number with personalized scripting and call tracking
- Customizable member welcome booklet and materials
- White-labeled formulary and marketing materials

Clinical flexibility

- Full portfolio of custom formularies with an optimized formulary shield
- Clinical programs that advance the overall DTE strategy
- More drug savings by moving members to lower-cost alternatives
- Customize or layer on utilization management programs

Custom client support

- Specialized call center capabilities
- Fully customizable plan designs
- Operational access to the platform
- Processes customized to promote the DTE strategy

STOP-LOSS INSURANCE

The KBA stop-loss experience

20+

stop-loss underwriters and carriers

$150M+

annual stop-loss premium placed

Comprehensive pricing and medical service analysis for every DTE plan ensures proper pricing. KBA provides the administration the stop-loss markets require:

- ✓ Bordereau reporting
- ✓ Evaluation of networks
- ✓ Initial underwriting
- ✓ Plan design actuarial
- ✓ Claim reporting and reconciliation
- ✓ Care management
- ✓ Negotiation of rates and terms

CARRIER RELATIONSHIPS INCLUDE

Tokio Marine	Nationwide
Berkshire Hathaway	Sun Life
Liberty Mutual	Granular (Google)
Companion Life	Crum & Forster
American Fidelity	RGA



DTE PROGRAM MARKETING AND SALES

KBA works with you to manage and sell

- Branding and messaging
- Direct and brokerage sales distribution with a focused sales representative
- Brochures, website and media
- Stop-loss and level-funded options down to 20 employee lives
- Sell through established brokers in your service area



SALES STRATEGY

A community-based sales strategy

1. A locally based sales rep with strong community ties
2. Work with DTE provider partners to identify strategic employers for early adoption
3. Partner with select brokers whose incentives and goals are aligned
4. Shared pipeline reporting and collaboration with DTE provider partners
5. Marketing and promotion of the new option in the community

A complete marketing system

- Customized branding and messaging
- Customized brochures
- Customized website
- Targeted employer outreach
- Sales system: social media, email, text and more



Sample private-label plan website

COLLABORATION OF STAKEHOLDERS

Everyone at the table wins

Key Benefit Administrators

- Full TPA administration
- Stop-loss administration
- True transparency and analytics
- Population health management
- Utilization and case management
- Sales and marketing
- Transparent PBM
- Website and mobile app
- Customizable options and plans

OhioHealth's DTE

- Incremental revenue and margin
- Branding and messaging
- Benefit plan integration
- Network steerage
- Direct relationship with self-funded employers
- Referral authorization
- Plan transparency
- Direct access to members

Employer

- Reduced spend
- "At the table" with OhioHealth
- Improved analytics
- Complete transparency
- Lower stop-loss
- Benefit plan stabilization
- Healthier population

Employee

- Reduced out-of-pocket
- Improved health
- Better coordination and navigation of care
- Direct access to the best medical providers

Who does what

OhioHealth

- Tier 1 provider network
- Provider recruitment, if necessary
- Network pricing and contract terms
- Referral management protocols and approvals
- Medical director oversight
- Practice improvement program using KBA/AHDI data, with KBA assistance

KBA Consulting

- Program oversight and integration across the enterprise
- Goal development, agreement and tracking
- Start-up assessments: market, network, operations and readiness
- Draft business plan with financial projections
- Engagement with OhioHealth's C-suite, operations, HR, managed care, marketing, strategy and business development

KBA Operations

- Network evaluation: actuarial pricing
- PPO wrap and tertiary transplant network, if needed
- Claims administration
- Plan design development, testing and administration
- Employer, member and provider customer service
- Customized member, provider, agent and HR portal
- Billing, enrollment and premium management
- Fulfillment: SPD and customized ID cards
- COBRA/HIPAA administration
- Out-of-network negotiations
- Wrap PPO repricing
- Nurse coaching for the chronically ill
- Nurse triage to physicians and facilities
- Stop-loss underwriting, pricing and placement
- Stop-loss claims administration
- Complex case management
- PBM administration interface
- Reporting: utilization, gaps in care, physician profiling, referrals, chronic disease, employer risk
- Ancillary administration: dental, vision, life, 5500 and more
- HSA, FSA and HRA administration
- Sales and marketing plan: branding, distribution, sales management

PRE-IMPLEMENTATION

Get the network and the pricing right first

STEP 1

Provider file

- Determine gaps in medical service delivery
- Identify other key providers to potentially contract with on behalf of OhioHealth
- Wrap the network to cover all other provider types, using Key Preferred's broad statewide contracts

STEP 2

Pricing

- Dial in competitive stop-loss rates and OhioHealth's net revenue and margins
- Actuarial evaluation compared to the market

IMPLEMENTATION

From decision to go-live in 6 to 8 months

DECIDE

Choose a path

Branded plan (Path A) or join Key Preferred (Path B)

MONTH 1

Agreement and project scope

MONTHS 1-6

Implementation

Market outreach begins in month 3

MONTHS 3-6

Sales and marketing

Hire the sales executive

MONTHS 6-8

Go live

First employer groups enroll

Recommended next step: a working session with OhioHealth leadership to choose a participation path, confirm the preferred service area, and start the pre-implementation network and pricing analysis.

A partner you can trust

Our objectives are aligned

We know the Ohio market

Completely transparent

The sophistication and infrastructure to compete

Derek Moore
Senior Vice President, Key Benefit Administrators
845-283-9973 | derek.moore@keybenefit.com

KEY PREFERRED NETWORK

A

Appendix: financial projections and assumptions

Payer mix, margin, market share, steerage and scenario detail for the OhioHealth-branded plan (Path A)

KEY BENEFIT ADMINISTRATORS

Estimated OhioHealth 2023 payer mix

Payer Group	Total Inpatient Days	Revenue	Payer Mix
All OhioHealth	**562,263**	**$5,399,076K**	**100%**
Medicare	137,889	$631,316K	24%
Medicaid	33,175	$450,321K	6%
Other Government Pay	0	-	-
No-Pay, Charity Care	33,959	-$357,365K	6%
Commercial/Self Pay	357,240	$4,318,736K	64%

Values from the CMS Medicare Provider Cost Report, Definitive Healthcare and MedPAR.

Estimating payer mix

- Medicare, Medicaid and other government mixes are each group's inpatient days divided by total inpatient days.
- No-pay and charity care is no-pay/charity revenue as a share of total revenue.
- Commercial/self-pay is the remainder.

Estimating inpatient days

- No-pay and commercial days are their payer mix shares multiplied by total inpatient days.
- Outpatient payer mix is assumed to mirror inpatient. Total Medicare revenue is from MedPAR.

APPENDIX

Estimated margin: commercial vs. government pay

RAND data estimates OhioHealth commercial reimbursement at 300% of Medicare rates.

Commercial margin

Counterfactual: all government-pay patients paid at commercial rates

Commercial Revenue	$4,318,736K
Gov-pay revenue at 300% current rate	$3,243,260K
Total Revenue	$7,561,996K
Total Costs	$5,032,600K
Net Income	**$2,529,396K**

33%

Government-pay margin

Counterfactual: all commercial patients paid at Medicare rates

Commercial Revenue Divided by 300%	$1,440,311K
Gov-pay revenue	$1,081,637K
Total Revenue	$2,521,949K
Total Costs	$5,032,600K
Net Income	**$-2,510,651K**

-100%

Estimating market share (1 of 3)

Hospital	Beds	Inpatient charges	Outpatient charges	IP share	OP share*
OhioHealth Riverside Methodist Hospital	766	$3,668,762.9K	$2,224,512.0K	15%	5%
OhioHealth Marion General Hospital	179	$360,019.4K	$715,340.7K	1%	2%
OhioHealth OBleness Hospital	67	$109,434.3K	$358,712.5K	<1%	1%
OhioHealth Grant Medical Center	551	$2,124,810.4K	$3,191,154.9K	8%	7%
OhioHealth Van Wert Hospital	27	$22,972.6K	$109,348.8K	<1%	<1%
OhioHealth Mansfield Hospital	240	$697,954.6K	$852,937.6K	3%	2%
OhioHealth Doctors Hospital	174	$579,743.1K	$675,616.4K	2%	1%
Berger Hospital	83	$86,914.6K	$324,065.7K	<1%	1%
OhioHealth Southeastern Medical Center	86	$78,920.3K	$218,351.9K	<1%	<1%
OhioHealth Grady Memorial Hospital	60	$85,483.2K	$383,263.2K	<1%	1%
OhioHealth Dublin Methodist Hospital	110	$427,777.3K	$738,508.7K	2%	2%
Morrow County Hospital	25	$5,287.3K	$55,753.7K	<1%	<1%
OhioHealth Hardin Memorial Hospital	30	$10,040.0K	$95,091.2K	<1%	<1%
OhioHealth Shelby Hospital	21	$19,874.9K	$64,353.2K	<1%	<1%
OhioHealth Rehabilitation Hospital	139	$169,877.4K	n/a	1%	n/a
OhioHealth Rehabilitation Hospital - Dublin	40	$39,117.4K	n/a	<1%	n/a
University Hospitals Samaritan Medical Center	28	$55,036.2K	$210,964.8K	<1%	<1%

34%

OhioHealth inpatient market share

22%

OhioHealth outpatient market share

OhioHealth facilities in green. *Outpatient share assumes 66% of local physicians are employed by area hospitals (Physician Advocacy Institute, Avalere Physician Employment Trends Study 2019-2023).

APPENDIX

Estimating market share (2 of 3)

Hospital	Beds	Inpatient charges	Outpatient charges	IP share	OP share*
Mount Carmel St Anns	263	$945,697.0K	$851,668.4K	4%	2%
Mount Carmel East	363	$2,229,620.2K	$1,560,111.1K	9%	3%
Genesis Hospital	252	$715,816.1K	$1,428,636.7K	3%	3%
Fairfield Medical Center	185	$458,109.7K	$746,201.0K	2%	2%
Ohio State University Wexner Medical Center	1,935	$4,469,341.2K	$4,885,446.7K	18%	11%
Chillicothe VA Medical Center	91	n/a	n/a	n/a	n/a
Memorial Hospital	54	$106,887.7K	$463,327.3K	<1%	1%
Coshocton Regional Medical Center	56	$67,823.7K	$93,247.9K	<1%	<1%
Adena Regional Medical Center	237	$974,117.6K	$1,293,716.6K	4%	3%
Madison Health	45	$30,658.5K	$230,683.2K	<1%	1%
The James Cancer Hospital and Solove Research Institute	356	$2,192,396.8K	$4,577,080.8K	9%	10%
Mount Carmel New Albany	60	$124,143.4K	$251,073.0K	<1%	1%
Diley Ridge Medical Center	10	$4,845.0K	$115,396.3K	<1%	<1%
Ontario Hospital	49	$69,894.8K	$332,013.0K	<1%	1%
Mount Carmel Dublin	60	n/a	n/a	n/a	n/a
Mercy Health - Willard Hospital	20	$14,890.1K	$91,208.8K	<1%	<1%
Bucyrus Hospital	25	$26,808.7K	$169,251.9K	<1%	<1%

34%

OhioHealth inpatient market share

22%

OhioHealth outpatient market share

OhioHealth facilities in green. *Outpatient share assumes 66% of local physicians are employed by area hospitals (Physician Advocacy Institute, Avalere Physician Employment Trends Study 2019-2023).

APPENDIX

Estimating market share (3 of 3)

Hospital	Beds	Inpatient charges	Outpatient charges	IP share	OP share*
Barnesville Hospital	25	$3,131.6K	$68,752.5K	<1%	<1%
Galion Hospital	25	$64,657.7K	$287,939.0K	<1%	1%
Wyandot Memorial Hospital	24	$17,681.0K	$177,955.6K	<1%	<1%
Hocking Valley Community Hospital	25	$16,381.3K	$108,231.4K	<1%	<1%
Select Specialty Hospital - Columbus	162	$738,619.1K	n/a	3%	n/a
Select Specialty Hospital - Columbus South	43	$278,135.0K	n/a	1%	n/a
Mount Carmel Rehabilitation Hospital	60	$47,868.2K	n/a	<1%	n/a
ClearSky Rehabilitation Hospital of Lancaster	36	$7,474.9K	n/a	<1%	n/a
Nationwide Childrens Hospital	700	$2,421,947.1K	$2,351,603.5K	10%	5%
Central Ohio Behavioral Healthcare - Columbus	260	n/a	n/a	n/a	n/a
Appalachian Behavioral Healthcare Hospital	92	n/a	n/a	n/a	n/a
Ohio Hospital for Psychiatry	130	$124,969.5K	$1,014.6K	<1%	<1%
Ridgeview Behavioral Hospital	84	$11,402.2K	n/a	<1%	n/a
Columbus Springs Dublin	144	$120,745.0K	$34,229.4K	<1%	<1%
SUN Behavioral Columbus	126	$64,680.0K	$7,355.6K	<1%	<1%
RiverVista Behavioral Health (RiverVista Vista Health and Wellness)	80	$116,500.8K	$443.2K	<1%	<1%
Mount Carmel Behavioral Health	80	$95,972.4K	$919.2K	<1%	<1%
Hickory Behavioral Center at Cambridge	80	$15,618.2K	n/a	<1%	n/a

34%

OhioHealth inpatient market share

22%

OhioHealth outpatient market share

OhioHealth facilities in green. *Outpatient share assumes 66% of local physicians are employed by area hospitals (Physician Advocacy Institute, Avalere Physician Employment Trends Study 2019-2023).

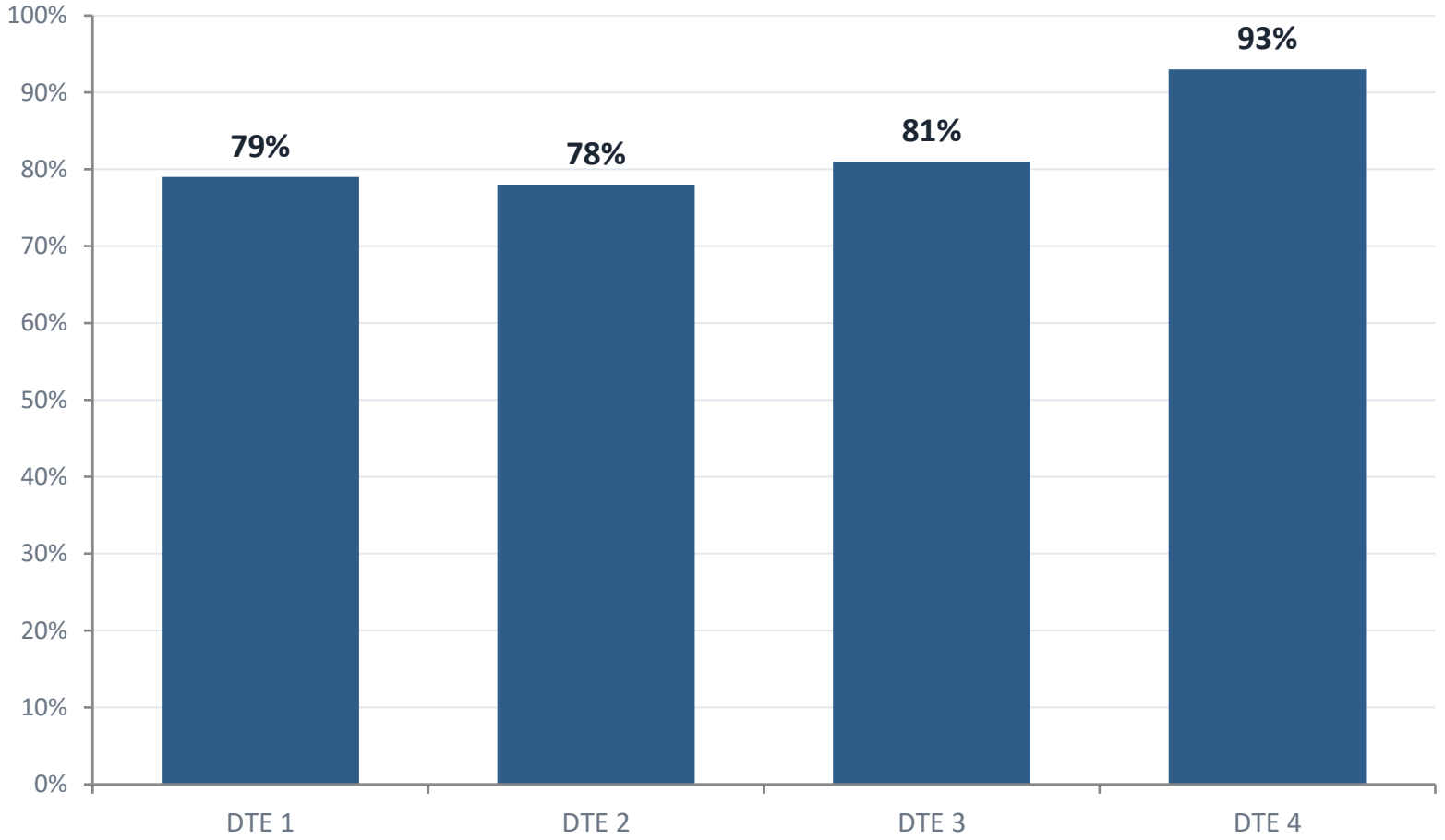
APPENDIX

OhioHealth hospital metrics

Hospital	Inpatient revenue	Outpatient revenue	Commercial revenue	Operating expenses	Net income	Commercial mix	Commercial margin
Riverside Methodist Hospital	$1,071,279K	$649,558K	$1,382,874K	$1,687,800K	$35,446K	63.9%	29.8%
Marion General Hospital	$107,286K	$213,172K	$249,351K	$267,801K	$52,234K	57.0%	41.9%
OBleness Hospital	$36,770K	$120,527K	$121,272K	$142,230K	$15,219K	56.2%	38.1%
Grant Medical Center	$563,075K	$845,656K	$1,155,279K	$1,294,110K	$113,175K	68.0%	32.3%
Van Wert Hospital	$8,431K	$40,131K	$33,813K	$57,659K	$-9,120K	61.1%	26.0%
Mansfield Hospital	$189,146K	$231,146K	$324,686K	$417,490K	$3,547K	57.9%	32.0%
Doctors Hospital	$151,313K	$176,336K	$235,153K	$328,377K	$-959K	63.4%	35.8%
Berger Hospital	$29,029K	$108,238K	$112,787K	$126,568K	$10,714K	64.4%	32.0%
Southeastern Medical Center	$26,596K	$73,585K	$73,509K	$94,662K	$5,390K	54.7%	38.2%
Grady Memorial Hospital	$29,235K	$131,076K	$137,081K	$140,820K	$19,635K	63.8%	32.0%
Dublin Methodist Hospital	$148,011K	$255,524K	$353,393K	$279,508K	$124,123K	70.9%	44.5%
Morrow County Hospital	$2,041K	$21,521K	$17,153K	$26,445K	$-2,880K	54.3%	27.3%
Hardin Memorial Hospital	$4,036K	$38,227K	$35,177K	$36,697K	$5,591K	49.4%	35.0%
Shelby Hospital	$7,930K	$25,677K	$28,610K	$39,462K	$-5,876K	46.0%	9.3%
Rehabilitation Hospital	$74,576K	$0K	$50,379K	$59,531K	$15,029K	70.5%	51.6%
Rehabilitation Hospital - Dublin	$19,950K	$0K	$8,222K	$33,438K	$-13,495K	53.4%	22.9%

Source: CMS provider cost reports; Medicare costs from MedPAR. Metrics labeled "estimated" are derived from other data points. Full metric set available on request.

Steerage rates for current DTE networks



83%

average Tier 1 steerage across KBA DTE networks

Data collected from KBA DTE networks in Q1 2024.

APPENDIX

Projected plan growth over 20 years

Year	Employees	Members	OhioHealth share of inpatient care	OhioHealth share of outpatient care
1	2,100	3,800	60%	50%
2	4,200	7,600	60%	50%
3	6,900	12,400	65%	55%
4	**10,400**	**18,700**	**67%**	**58%**
5	14,700	26,500	68%	60%
6	20,200	36,400	68%	60%
7	26,700	48,100	69%	61%
8	34,300	61,700	69%	62%
9	42,900	77,200	69%	62%
10	52,200	94,000	70%	63%
11	61,800	111,200	70%	63%
12	71,100	128,000	70%	64%
13	79,900	143,800	71%	64%
14	87,600	157,700	71%	65%
15	94,200	169,600	71%	66%
16	99,700	179,500	72%	66%
17	103,900	187,000	72%	66%
18	107,200	193,000	72%	67%
19	109,800	197,600	72%	67%
20	111,600	200,900	72%	67%

- KBA estimates OhioHealth holds 34% inpatient and 22% outpatient market share in its service area today.
- Based on prior DTE experience, KBA expects a significant jump in steerage to OhioHealth.
- More steerage comes from member bonus benefits and the Convenience Card.
- High estimates start about 10% above target and low estimates about 10% below.
- Year 4 (highlighted) is the basis for the Year 4 projections.

APPENDIX

Year 4 projected revenue and profit

Pre-DTE cohort of 18,600 members	Inpatient	Outpatient	Professional	Total
Revenue†	$20,092,991	$23,531,205	$10,988,873	**$54,613,069**
Net Income‡	$5,304,550	$6,212,238	$2,901,062	**$14,417,850**

† Proportional revenue from 34% inpatient and 22% outpatient market share. ‡ Commercial margin of 26.4% (80% of the estimated commercial margin).

With DTE	Inpatient	Outpatient	Professional	Total
Revenue[o]	$39,921,658	$62,608,301	$29,237,544	**$131,767,503**
Net Income*	$10,539,318	$16,528,591	$7,718,712	**$34,786,621**

[o] 67% inpatient and 58% outpatient steerage based on KBA DTE experience. * 26.4% commercial margin, less 10 points for competitive rates, plus 10 points for lower billing costs and better patient collections via the Convenience Card.

Year 4 profit bridge

Total Increase in Net Income	$20,368,771
Network Access Fee (Revenue) of $1 PMPM	$223,200
Total Profit Growth	$20,591,971
Minus Implementation Fee (One-Time)	-$0
Minus Sales and Marketing (Annual – Ongoing)	-$180,000
Total	**$20,411,971**

APPENDIX

Target scenario detail by service line ($K)

Target estimate: 50%+ within-plan steerage, 26.4% commercial margin, modeled with Year 1 fees and steerage

Metric	500 lives	1000 lives	5000 lives	25000 lives
Pre-DTE Group Inpatient Revenue	$540.1	$1,080.3	$5,401.3	$27,006.7
Pre-DTE Group Inpatient Margin	$142.6	$285.2	$1,426.0	$7,129.8
DTE Group Inpatient Revenue	$959.0	$1,918.1	$9,590.3	$47,951.3
DTE Group Inpatient Margin	$253.2	$506.4	$2,531.8	$12,659.1
DTE Group Incremental Inpatient Revenue	$418.9	$837.8	$4,188.9	$20,944.5
DTE Group Incremental Inpatient Margin	$110.6	$221.2	$1,105.9	$5,529.4
Pre-DTE Group Outpatient Revenue	$632.6	$1,265.1	$6,325.6	$31,628.0
Pre-DTE Group Outpatient Margin	$167.0	$334.0	$1,670.0	$8,349.8
DTE Group Outpatient Revenue	$1,453.2	$2,906.3	$14,531.7	$72,658.6
DTE Group Outpatient Margin	$383.6	$767.3	$3,836.4	$19,181.9
DTE Group Incremental Outpatient Revenue	$820.6	$1,641.2	$8,206.1	$41,030.6
DTE Group Incremental Outpatient Margin	$216.6	$433.3	$2,166.4	$10,832.1
Pre-DTE Group Professional Revenue	$295.4	$590.8	$2,954.0	$14,770.0
Pre-DTE Group Professional Margin	$78.0	$156.0	$779.9	$3,899.3
DTE Group Professional Revenue	$678.6	$1,357.2	$6,786.2	$33,930.9
DTE Group Professional Margin	$179.2	$358.3	$1,791.6	$8,957.8
DTE Group Incremental Professional Revenue	$383.2	$766.4	$3,832.2	$19,161.0
DTE Group Incremental Professional Margin	$101.2	$202.3	$1,011.7	$5,058.5
Net Margin Less DTE Fees	**$104.4**	**$538.8**	**$4,014.0**	**$21,389.9**

APPENDIX

Target scenario summary ($K)

Target estimate: 50%+ within-plan steerage, 26.4% commercial margin, modeled with Year 1 fees and steerage

Metric	500 lives	1000 lives	5000 lives	25000 lives
Pre-DTE Group Total Revenue	$1,468.1	$2,936.2	$14,680.9	$73,404.7
Pre-DTE Group Total Margin	$387.6	$775.2	$3,875.8	$19,378.8
DTE Group Total Revenue	$3,090.8	$6,181.6	$30,908.2	$154,540.8
DTE Group Incremental Total Revenue	$1,622.7	$3,245.4	$16,227.2	$81,136.1
Incremental Total Revenue by Member	$3.2	$3.2	$3.2	$3.2
DTE Group Total Margin	$816.0	$1,632.0	$8,159.8	$40,798.8
DTE Group Incremental Total Margin	$428.4	$856.8	$4,284.0	$21,419.9
Wellness Share	$6.0	$12.0	$60.0	$300.0
Implementation Fee (One-Time)	$150.0	$150.0	$150.0	$150.0
Sales and Marketing Annual Fee	$180.0	$180.0	$180.0	$180.0
Net Margin Less DTE Fees	**$104.4**	**$538.8**	**$4,014.0**	**$21,389.9**

APPENDIX

High scenario summary ($K)

High estimate: 60%+ within-plan steerage, 31.4% commercial margin, modeled with Year 1 fees and steerage

Metric	500 lives	1000 lives	5000 lives	25000 lives
Pre-DTE Group Total Revenue	$1,468.1	$2,936.2	$14,680.9	$73,404.7
Pre-DTE Group Total Margin	$387.6	$775.2	$3,875.8	$19,378.8
DTE Group Total Revenue	$3,676.9	$7,353.9	$36,769.3	$183,846.5
DTE Group Incremental Total Revenue	$2,208.8	$4,417.7	$22,088.4	$110,441.9
DTE Group Total Margin	$1,154.6	$2,309.1	$11,545.6	$57,727.8
DTE Group Incremental Total Margin	$767.0	$1,534.0	$7,669.8	$38,349.0
Wellness Share	$6.0	$12.0	$60.0	$300.0
Implementation Fee (One-Time)	$150.0	$150.0	$150.0	$150.0
Sales and Marketing Annual Fee	$180.0	$180.0	$180.0	$180.0
Net Margin Less DTE Fees	**$443.0**	**$1,216.0**	**$7,399.8**	**$38,319.0**

APPENDIX

Low scenario summary ($K)

Low estimate: 40%+ within-plan steerage, 21.4% commercial margin, modeled with Year 1 fees and steerage

Metric	500 lives	1000 lives	5000 lives	25000 lives
Pre-DTE Group Total Revenue	$1,468.1	$2,936.2	$14,680.9	$73,404.7
Pre-DTE Group Total Margin	$387.6	$775.2	$3,875.8	$19,378.8
DTE Group Total Revenue	$2,505.4	$5,010.8	$25,054.0	$125,270.0
DTE Group Incremental Total Revenue	$1,037.3	$2,074.6	$10,373.1	$51,865.3
DTE Group Total Margin	$536.2	$1,072.3	$5,361.6	$26,807.8
DTE Group Incremental Total Margin	$148.6	$297.2	$1,485.8	$7,428.9
Wellness Share	$6.0	$12.0	$60.0	$300.0
Implementation Fee (One-Time)	$150.0	$150.0	$150.0	$150.0
Sales and Marketing Annual Fee	$180.0	$180.0	$180.0	$180.0
Net Margin Less DTE Fees	**$-175.4**	**$-20.8**	**$1,215.8**	**$7,398.9**